

1. Administrative & Study Identification

Study Title: Study of Izalontamab Brengitecan (BMS-986507) Versus Platinum-Pemetrexed for EGFR-mutated Non-small Cell Lung Cancer After Failure of EGFR TKI Therapy (IZABRIGHT-Lung01) **Official Title:** IZABRIGHT-Lung01: A Randomized, Open-label, Phase 2/3 Study of Izalontamab Brengitecan (BMS-986507) Versus Platinum-based Chemotherapy in Patients With EGFR-mutated Non-small Cell Lung Cancer and Disease Progression on EGFR Tyrosine Kinase Inhibitor Therapy **Short Title/Acronym:** IZABRIGHT-Lung01 **Protocol Number:** CA244-0010 **NCT Number:** NCT07100080 **Posting ID:** 1105109543125136458 **Trial Status:** RECRUITING (Currently recruiting participants) **Sponsor/Company:** Bristol Myers Squibb (BMS) **Investigational Product:** Izalontamab Brengitecan (BMS-986507 / Iza-bren) - EGFRxHER3 Bispecific Antibody-Drug Conjugate (ADC) **Phase of Development:** PHASE2, PHASE3 **Medical Monitor:** Clinical Operations Lead, Oncology, Bristol Myers Squibb

2. Study Synopsis & Rationale

2.1 Study Objective & Summary Summary: A Study of Izalontamab Brengitecan (BMS-986507) versus Platinum-Pemetrexed for EGFR-mutated Non-small Cell Lung Cancer after failure of EGFR TKI Therapy. **Primary Objective:** - *Phase 2:* To determine the Recommended Phase 3 Dose (RP3D) / Dose-Limiting Toxicities (DLTs) of BMS-986507. - *Phase 3:* To evaluate the Progression-Free Survival (PFS) Assessed by Response Evaluation Criteria in Solid Tumors version 1.1 (RECIST v1.1) per blinded independent central review (BICR) of Izalontamab Brengitecan versus platinum-pemetrexed chemotherapy. **Secondary Objectives:** To evaluate Objective Response (OR) Assessed by RECIST v1.1 per BICR, Progression-Free Survival (PFS) Assessed by RECIST v1.1 per BICR, and Overall Survival (OS).

2.2 Background & Rationale To address the gap in treatment options for patients with locally advanced or metastatic non-squamous non-small cell lung cancer (NSCLC) harboring EGFR mutations whose disease has progressed following treatment with a 3rd-generation EGFR tyrosine kinase inhibitor (TKI) (e.g., osimertinib). Standard salvage therapies yield limited benefits, establishing a high unmet medical need. Izalontamab Brengitecan (Iza-bren) is a novel first-in-class bispecific ADC targeting both EGFR and HER3, designed to overcome resistance mechanisms and deliver cytotoxic payload directly to tumor cells.

3. Study Design & Methodology

3.1 Design Framework Study Type: INTERVENTIONAL **Control Type:** Active Comparator (Platinum-Pemetrexed chemotherapy) **Blinding:** Open-label **Randomization:** Randomized **Status:** RECRUITING

3.2 Planned Enrollment & Duration Targeted Enrollment: 596 evaluable patients **Number of Sites:** Approximately 186 global locations **Estimated Study Start:** 05-Nov-2025 **Estimated Primary Completion:** 15-Sep-2028

4. Subject Selection (Eligibility Criteria)

Criteria Type: PHASE_REQUIREMENT (Trial must be in PHASE2, PHASE3)

4.1 Inclusion Criteria 1. Non-squamous NSCLC, not amenable to treatment in curative intent. 2. Documented evidence of EGFR mutation (exon 19 deletion, L858R mutation). 3. Progressive disease on a 3rd-generation (such as osimertinib, furmonertinib, lazertinib, ...) EGFR-TKI-based mono- or combination therapy regimen as the most recent line of therapy in an adjuvant, locally advanced, or metastatic treatment setting. 4. Eligible to receive a platinum-based doublet chemotherapy regimen (either cisplatin or carboplatin in combination with pemetrexed).

4.2 Exclusion Criteria 1. Inadequate organ function and/or bone marrow reserve. 2. Leptomeningeal metastases or spinal cord compression. 3. Poorly controlled systemic medical conditions. 4. Other protocol-defined inclusion/exclusion criteria apply.

5. Intervention & Treatment Plan

5.1 Investigational & Comparator Regimens Investigational Dosage/Frequency: Izalontamab Brengitecan administered in Phase 2 dose-finding cohorts to determine the optimal dose, followed by the selected Recommended Phase 3 Dose (RP3D). Administered on Day 1 of specified cycles. **Comparator Regimen:** Platinum-pemetrexed chemotherapy (utilizing either carboplatin or cisplatin). *Comparator Drug Details (Cisplatin):* - **Drug Preferred Name:** cisplatin - **Trade Names:** Platinol - **ATC Code:** L01XA01 **Route of Administration:** Intravenous (IV) Infusion. **Duration of Treatment:** Cycles continue until disease progression, unacceptable toxicity, or withdrawal of consent.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care for managing chemotherapy-related adverse events (e.g., anti-emetics, routine premedications). **Prohibited:** Concomitant systemic anti-cancer therapies, investigational agents, or live vaccines during the trial.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Tumor biopsy/mutation testing, Baseline Imaging (CT/MRI), Safety Labs
Baseline	Day 0	Randomization, First Dose Administration
Interim	Every 3 weeks (Q3W)	Safety Labs, Treatment administration, AE monitoring; Tumor imaging every 6-8 weeks per RECIST 1.1
End of Study	Post-Progression	Final Vital Signs, Exit Interview, IP Accountability, Survival Follow-up

7. Outcome Measures (Endpoints)

7.1 Primary Outcomes 1. Recommended Phase 3 Dose (RP3D) of BMS-986507: Phase 2 2. Progression-Free Survival (PFS) Assessed by Response Evaluation Criteria in Solid Tumors version 1.1 (RECIST v1.1) per blinded independent central review (BICR): Phase 3

7.2 Secondary Outcomes 1. Objective Response (OR) Assessed by RECIST v1.1 per BICR 2. Progression-Free Survival (PFS) Assessed by RECIST v1.1 per BICR 3. Overall Survival (OS)

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous collection via HCP clinical assessments, focusing closely on cytopenias, gastrointestinal toxicities, and potential interstitial lung disease (ILD). **Serious Adverse Events (SAEs):** Reported within 24 hours of site awareness to BMS Pharmacovigilance. **Data Monitoring Committee (DMC):** Independent Data Monitoring Committee (IDMC) reviews safety and efficacy data, notably during the Phase 2/3 dose-selection transition.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for all efficacy evaluations (PFS, OS, OR). Safety Set for all subjects who receive at least one dose of study medication. **Sample Size Justification:** Target Enrollment = 596 appropriately powered to detect a statistically significant improvement in PFS over standard-of-care platinum doublet chemotherapy. **Handling of Missing Data:** Censoring rules applied per standard FDA/EMA oncology guidelines for time-to-event endpoints (Kaplan-Meier estimates).

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Routine onsite and remote monitoring visits to verify source data, regulatory document compliance, and IP accountability. **Audit Trail:** Validated eCRF system with comprehensive audit trails, electronic signatures, and active data clarification form (DCF) generation.

11. Study Identifiers & Governance

Primary ID: CA244-0010 **Registry Number:** NCT07100080 **Posting ID:** 1105109543125136458 **Sponsor/Lead Organization:** Bristol Myers Squibb **Study Title:** Study of Izalontamab Brengitecan (BMS-986507) Versus Platinum-Pemetrexed for EGFR-mutated Non-small Cell Lung Cancer After Failure of EGFR TKI Therapy (IZABRIGHT-Lung01) **Trial Status:** RECRUITING

12. Clinical Framework (Oncology Specific)

Disease Area / Condition: Non-Small Cell Lung Cancer **Name:** Metastatic Non Small Cell Lung Cancer **Preferred Name:** Metastatic non-small cell lung cancer **Preferred UMLS Name:** EGFR-mutated Non-Small Cell Lung Cancer **Semantic Type:** Neoplastic Process **Diagnosis Threshold / Definition:** Histologically/cytologically confirmed non-squamous NSCLC with EGFR exon 19 deletion or L858R mutation. Stage IV includes: (Any T, Any N, M1a); (Any T, Any N, M1b). M1a: Lung cancer with separate tumor nodule(s) in a contralateral lobe; tumor with pleural nodules or malignant pleural (or pericardial) effusion. M1b: Distant metastasis. (AJCC 7th ed.) **MeSH Code:** D016938 **SNOMED ID:** 457721000124104 **Medical Methodology:** Targeted biologic therapy vs. standard cytotoxic chemotherapy. **Optimization Target:** Progression-Free Survival (PFS) and Disease Control.

13. Study Procedures & Methodology

Management Pathway: 2nd-line treatment or beyond following failure of 3rd-generation EGFR-TKI. **Education Protocol (HCP):** Investigator meeting and site initiation visits (SIV) focusing on proper ADC handling, administration, and AE management. **Education Protocol (Patient):** Informed consent process, counseling on potential infusion reactions, cytopenias, and reporting of emergent symptoms. **Quality Audit Mechanism:** Routine sponsor audits and rigorous BICR monitoring for unbiased radiological reads.

14. Observation & Follow-Up Schedule

Total Duration: Active treatment until disease progression; long-term survival follow-up thereafter (estimated ~5 years). **Onsite Visit Cadence:** Every 3 weeks (Q3W) for dosing and safety checks. **Remote Monitoring Frequency:** Between treatment cycles via phone follow-up for safety/AE status as needed. **Checklist Review Interval:** Radiological tumor assessments every 6-8 weeks until disease progression.

15. Sign-off & Approval

Role	Name	Signature	Date		----	----	----	----
Principal Investigator	[Insert PI Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Insert BMS Lead Name]	_____	[DD-MMM-YYYY]					
Lead Statistician	[Insert Statistician Name]	_____	[DD-MMM-YYYY]					